



PROCEDURE

Transport of Mental Health Consumers

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	NMHS Mental Health – Inpatient Adult Mental Health, Community Adult Mental Health and Mental Health Specialities (excluding State Forensic Mental Health Services)

1. Aim

The aim of this procedure is to ensure that all mental health consumers have access to transport services that are timely and appropriate. Such transport that falls within the scope of this procedure relates to a consumer being conveyed from the community to a hospital, between hospital sites, from a hospital for treatment activity or between community settings.

This procedure outlines requirements for all staff/workers within NMHS MH Services and should be read in conjunction with [MP0099/18 Community Mental Health Status Assessment: Role of Mental Health Clinicians Policy](#).

2. Risk

Transporting consumers, whilst an essential part of care, may be associated with increased risks that require assessment and mitigation in advance. Specific procedures are required to avoid the risks of consumers' care being delayed, and staff or others' safety being compromised.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Principles

Transporting consumers must be undertaken in a manner that is least restrictive and safe for both the consumer and staff and maintains the safety of both consumers, escorting staff and the public.

The decision to transport a consumer is a clinical decision and the reason for the transportation must be fully documented including the relevant risk assessment. Please refer to the [NMHS Safe Transport of Individuals with a Mental Illness Guideline](#).

4. Reasons For Transporting Consumers

At times it may be clinically appropriate to provide or arrange transport for consumers to facilitate their assessment, treatment, rehabilitation and on cultural grounds.

The reasons for transporting consumers may include but are not limited to:

- Transfer of a voluntary inpatient to another health facility.
- Transfer of an involuntary consumer under the [Mental Health Act \(WA\) 2014](#).
- Transport of a consumer as part of group or individual treatment activity.
- To facilitate assessment and/or admission as a voluntary or involuntary consumer.
- To support attendance at outpatient appointments.
- Other statutory reasons, including court appearances.

Refer to the NMHS [Mental Health Transport](#) HUB page for contractor numbers.

5. Procedures

- When requesting mental health transport for involuntary patients a [MH Patient Transport Risk Rating Form](#) must be completed and a copy provided to the Transport Officers and WA Police with the Form 4A or Form 7D:
 - [Safe Transport of individuals with a mental illness Transport Orders \(Form 4A\) Guideline](#)
 - [Safe Transport of individuals with a mental illness Apprehension and Return Orders \(Form 7D\) Guideline](#)
 - [Information for Referring Practitioners](#)
- There must be a current [MH Patient Transport Risk Rating Form](#) and review of any changes in mental state and risks by the clinical treating team prior to any transport. Transport mode and staff requirements (a minimum of one) for transfer will be guided by the risk assessment.
- Staff of mental health services should establish whether the consumer is willing to be transported before initiating this as this will inform risk assessment, noting that the assessment may change prior to the escort/transport occurring.
- Only those consumers identified as low transport risk and consenting to the transport are to be transported in a government vehicle.
- Following completion of the transportation, any issues arising from the planned or unplanned transportation will be documented in the consumer's medical record
- All risks identified during the routine transport of a consumer must be reported and escalated as appropriate.

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- The gender and cultural needs of the consumer and of the staff involved in the transportation must be given careful consideration. This is especially important in relation to consumers who are:
 - Minors
 - [Aboriginal Consumers](#)
 - [Torres Strait Islander Health](#)
 - [Culturally and Linguistically Diverse](#) (CALD) Communities
 - [LGBTIQ+ Community](#)

5.1 Transport by mental health service vehicle

Where the consumer is known to the treating team, and there is no clinical or safety requirement for an ambulance, the use of a government vehicle to transport a consumer from the community to an emergency department or inpatient service may be appropriate.

When deciding whether to use a government vehicle for transport, staff must consider:

- the consumer's current mental state, especially their risk of unpredictable or erratic behavior.
- the consumer's previous transport history.
- the consumer's understanding and acceptance of the purpose and destination of the transport.
- the distance to be travelled and the time of the day.
- the escorting staff's knowledge of the consumer and their history.
- the number of escorts required.
- placement of the consumer in the vehicle should be in the rear seat next to the clinician, not behind the driver.
- the consumer's willingness to be transported by the proposed driver; and
- the safety needs of **all** persons travelling in the vehicle.

For low - medium acuity patients, non-emergency inter hospital transport in metropolitan WA refer to the [PaTCH](#) Transport Coordination Hub (PaTCH) for booking, scheduling and [transport information](#).

5.2 Use of Ambulance, Transport Officer, or Police Services

- A client who has been assessed and deemed to meet the criteria for the [Mental Health Act \(WA\) 2014](#) should **NOT** be transported in a government vehicle. Wilson Medic One (WMO) and or Police should be contacted to facilitate the transport.
- Staff must use endorsed risk assessment tools, (RAMP (SMHMR905) and risk assessment matrix (attached to a form 4A) for completing all required risk assessments and documentation to determine the use of either WMO and/or Police.
- If a consumer is **NOT** under the [Mental Health Act \(WA\) 2014](#) but requires assistance to attend hospital from a Community Mental Health clinic - and not deemed low risk then consider the use of St John Ambulance interhospital transport service, Wilson Medic One interhospital transport service, or National Patient Transport Interhospital transport. (contact numbers are available on the [NMHS Mental Health Transport HUB page](#)).
- In all instances of high acuity, life threatening physical health concerns **contact 000** for a St John Ambulance.

Refer to:

- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [MP 0165/21 Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Policy](#)
- [MP 0063/17 Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy](#)
- [NMHS MH Clinical Observation Procedure](#)

5.3 Use of Taxi Services within Community Mental Health

NMHS Community Mental Health (CMH) recognises that consumers/carers may experience difficulties accessing transport to attend clinic-based appointments. To enhance access to our services, taxi vouchers may be issued to eligible consumers/carers. All staff must ensure they comply with this procedure to be responsible users of government monies, whilst also providing support to consumers/carers.

For each occasion a taxi voucher is requested or recommended, the responsible clinician should determine whether the consumer/carer is eligible and in need of a taxi voucher to attend an appointment, and whether this is for a one-way or return trip. Vouchers can be provided for a one-way trip, with consumers/carers encouraged to make their own way home after the appointment if possible.

5.3.1 Taxi Voucher Eligibility

The consumer/carer must fulfil all the eligibility criteria:

- A consumer/carer is unable to utilise public transport and all other options have been explored, such as a support person providing transport, changing appointment time/date to suit transport availability, transport training, use of taxi through Taxi Users' Subsidy Scheme (TUSS).
- There is a level of risk from the consumer/carer failing to attend the scheduled appointment
- The consumer/carer exhibits a significant level of functional impairment preventing the use of alternative transport options

Each allocation of a taxi voucher must be:

- Approved by the Consultant Psychiatrist in the consumer/carer's treating team. If the Consultant is unavailable, approval is to be sought from the Head of Clinical Services (or if unavailable, the Service Coordinator).
- Rationale and approval to be documented in the medical record by the requesting clinician

Refer to administration staff for Taxi Voucher / Cab-Charge Booking process on booking and recording.

5.4 Escorts

Mental health professionals are not required to accompany the patient during transportation by ambulance, however some consumers will require an escort to travel with them to ensure adequate support.

An escort may be a family member, patient support person, a mental health professional or a police officer. This will be determined by the consumer's presentation, needs and risks.

Aboriginal Health Liaison Officers may support:

- Acute patients, supported only in partnership with clinical staff and allied health.
- Hospital Extended Care Services (HECS) patients, support can be provided if a risk assessment is completed by the treating team is a low to medium risk and on cultural grounds only.

5.5 Inclusion of Carers, Close Family Members or other Personal Support Persons

If, in the clinician's professional judgement, it may be beneficial to have a carer, family member or other personal support person accompany the consumer e.g., from home to the health service (for assessment or admission), that support person may be invited to accompany them if they wish.

6. Safety Requirements Whilst Travelling in Vehicle

- Staff transporting consumers (and anyone accompanying them) must ensure that all passengers are appropriately restrained with a fitted seat belt and use of child proof lock (standard in government vehicles).
- Minors still requiring the use of a child restraint/car seat are not permitted to be transported in the NMHS Adult community mental health services government vehicles. Alternative options must be considered.
- When it is considered that it is safe to transport the consumer in a government vehicle and there is a clinician escort, the consumer should be seated in the rear passenger-side seat with the escort in the rear driver's side seat.
- All staff **must** always carry a [Sofi-hub \(Personal Duress Alarm\)](#) and a charged mobile phone when in the community, including when transporting a consumer. There must be clear documentation - on the staff whereabouts board/file - of the staff members intended location, anticipated return time and name and contact of the consumer they will be visiting within the community.
- Where a patient absconds while being transported staff should refer to:
 - [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#) and [Procedures](#)
 - [MP 0122/19 Clinical Incident Management Policy](#)
 - [NMHS MH Missing or Suspected Missing Patients Procedure](#)
 - [MH Duress Alarms Policy](#) (currently under review)
 - [MHPHDS Home and Community Visiting Policy](#) (currently under review)



- Patient Treatment, Support and Discharge Plan (TSDP)

6.1 Reporting of Incidents or Accidents:

Any incidents or accidents that occur during transportation should be managed and reported in accordance with protocols and procedures as per the on the [Datix Clinical Incident Management System \(Datix CIMS\)](#) as per the [NMHS Clinical Incident Management Policy and Procedure](#).

Staff may require emotional/psychological support post incidents. This should be offered by their line manager and can be accessed through the Employee Wellbeing Psychologist and the Employee Assistance Program. Where Mental health services staff that require support, refer to the [NMHS Clinical Incident Process Health Point - Debriefing Process](#).

7. Compliance and Evaluation

Each NMHS MH Site and service manager is to ensure compliance with this policy. Events requiring compliance monitoring may include the following:

- Incident or near miss event during the patient transportation process.
- Incidences of patient absconding (whilst under transport conditions)
- Incidents of serious assault by staff or patient (during transport).

These events would be reported as per required incident reporting in any clinical circumstance. Mental Health Services are required to report to the Office of the Chief Psychiatrist (OCP) [Reporting Notifiable Incidents – Public Mental Health Services](#).

Related internal policies, procedures and guidelines

DoH

[MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)

[MP 0165/21 Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Policy](#)

[MP 0063/17 Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders](#)

[MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy and Procedures](#)

[MP 0122/19 Clinical Incident Management Policy](#)

NMHS

[NMHS Motor Vehicle Fleet Policy](#)

Mental Health

[NMHS MH Clinical Observation Procedure](#)

[NMHS MH Case Management Policy](#)

[NMHS MH Clinical Risk Assessment Management \(CRAM\) Policy](#)
[NMHS MH Missing or Suspected Missing Patients Procedure](#)

References

[Chief Psychiatrist's Standards for Clinical Care](#)
[Mental Health Act 2014](#)
[Mental Health Act 2014 – Information for Referring Practitioners](#)
[Mental Health Regulations 2015](#)
[Department of Transport Children's seat belts and child restraints requirements.](#)
[NSQHSS – User Guide for Aboriginal and Torres Strait Islander Health](#)

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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Chief Psychiatrist's Standards for Clinical Care: Care Planning & Transfer of Care				
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The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative.

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